



# **ACCOUNTING POLICY PAPER LEVEL OF AGGREGATION**

## **SOCIAL HEALTH AUTHORITY**

22<sup>nd</sup> May 2026

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IFRS 17 Insurance Contracts establishes principles for the recognition, measurement, presentation and disclosure of insurance contracts within the scope of the Standard. The objective of IFRS 17 is to ensure that an insurer provides relevant information that faithfully represents those contracts. This information gives a basis for users of financial statements to assess the effect that insurance contracts have on the insurer's financial position, financial performance and cash flows.

# DOCUMENT TRACKING

## Document History

| Version | Author                                           | Date                       | Revision |
|---------|--------------------------------------------------|----------------------------|----------|
| 1.0     | Kenbright Actuarial & Financial Services Limited | 23 <sup>rd</sup> June 2025 | Creation |
| 2.0     | Kenbright Actuarial & Financial Services Limited | 22 <sup>nd</sup> May 2026  | Final    |

## This Document Has Been Submitted To

| Submitted to            | Date Submitted            |
|-------------------------|---------------------------|
| Social Health Authority | 22 <sup>nd</sup> May 2026 |

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## OUR REF

Social Health Authority \IFRS 17 Implementation Support\Social Health Authority– Accounting Policy Paper – Level of Aggregation\_(220526)

## DOCUMENT PURPOSE

Submission to the Management of Social Health Authority.

## DOCUMENT OWNERSHIP

The owner of this document is Social Health Authority.

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## ABBREVIATIONS & DEFINITIONS

| NAME        | DEFINITION                                 |
|-------------|--------------------------------------------|
| <b>CSM</b>  | Contractual Service Margin                 |
| <b>GMM</b>  | General Measurement Model                  |
| <b>IASB</b> | International Accounting Standards Board   |
| <b>IFRS</b> | International Financial Reporting Standard |
| <b>LoA</b>  | Level of Aggregation                       |
| <b>LRC</b>  | Liability for Remaining Coverage           |
| <b>PAA</b>  | Premium Allocation Approach                |
| <b>SHA</b>  | Social Health Authority                    |
| <b>VFA</b>  | Variable Fee Approach                      |

# 1. INTRODUCTION

## Background

- 1.1. The International Accounting Standards Board ('IASB') introduced IFRS 17 in May 2017, the standard modifies how entities identify, quantify, present, and disclose insurance contracts.
- 1.2. The Level of Aggregation ('LoA') is a crucial component of this new standard that determines how precisely insurance contracts are classified for financial reporting needs.
- 1.3. IFRS 17 seeks to improve the accuracy of revenue recognition and liability measurement by enforcing the LoA criteria. This will give stakeholders a better understanding of the risk profile and financial stability of an insurer.
- 1.4. The Social Health Authority ('SHA') is a statutory agency established under Section 4 of the Social Health Insurance Act, which was enacted on 22<sup>nd</sup> November 2023 to succeed the National Hospital Insurance Fund (NHIF) as Kenya's primary health coverage provider. NHIF ceased its operations on **30<sup>th</sup> June 2024**, with its final admissions date being 30<sup>th</sup> September 2024. The transition period, spanning from November 2023 to mid-2024, involved the legal transfer of assets, liabilities and functions. SHA officially commenced its operations on **1<sup>st</sup> October 2024**.
- 1.5. SHA is mandated to pool contributions from individuals, employers, and the government in order to purchase healthcare services from empaneled and contracted healthcare providers and ensure access to healthcare for registered members and their beneficiaries.
- 1.6. SHA operates under the Social Health Insurance Act and manages four funds:
  - a) Primary Healthcare Fund;
  - b) Social Health Insurance Fund (SHIF);
  - c) Emergency, Chronic, and Critical Illness Fund (ECCIF); and
  - d) Public Officers Medical Scheme Fund (POMSF).
- 1.7. Managing these funds exposes SHA to insurance risk; therefore, the Level of Aggregation guidance under IFRS 17 must be applied to appropriately group contracts for recognition and measurement.

## Purpose

- 1.8. This policy paper sets out the approach that Social Health Authority (SHA) will adopt in applying the Level of Aggregation requirements of IFRS 17 to its insurance contracts.
- 1.9. By establishing clear guidelines for aggregation, SHA aims to ensure more accurate sustainability analysis, better identification of onerous contracts, and improved decision-making processes.
- 1.10. The aggregation of insurance contracts into appropriate groups will allow the Authority to align its financial reporting with the latest accounting standards, ensuring transparency and consistency.
- 1.11. Distinguishing contracts based on their profitability levels and the potential risk of becoming onerous, SHA will be able to manage and mitigate risks within its portfolio.
- 1.12. The policy paper will outline the criteria for contract grouping as well as the measurement models used.

## Objective

- 1.13. This accounting policy paper will act as a guide to SHA on how to aggregate contracts which will, in turn, improve the accuracy of their financial reporting.

## Scope

- 1.14. The contents of this policy paper apply to all insurance contracts issued by SHA.

## 2. LEVEL OF AGGREGATION CRITERIA

- 2.1. The LoA criteria will aid in grouping of individual insurance contracts to recognize losses when a group of contracts is onerous and to determine the timing of recognition of profits related to a group of profitable contracts.
- 2.2. SHA is required to carry out the aggregation on initial recognition for all the contracts defined under the scope of IFRS 17 Insurance Contracts using substantial and verifiable information.
- 2.3. To ensure accurate and transparent financial reporting, the insurer's grouping process will involve:

### *a) Identification of Portfolio*

- 2.4. A portfolio shall comprise of contracts that are subject to similar risks and managed together. Therefore, portfolios within SHA will be based on the four distinct funds that SHA is mandated to operate which are:
  - The Primary Healthcare Fund
  - The Social Health Insurance Fund
  - Emergency, Chronic and Critical Illness Fund
  - Public Officers Medical Scheme Fund
- 2.5. These funds serve different healthcare coverage purpose, with different funding sources and risk profiles.

### *b) Subdivision of Portfolios*

- 2.6. Each of the portfolios above shall be subdivided based on expected profitability.

An entity shall divide a portfolio of insurance contracts issued into a minimum of:

- a) a group of contracts that are onerous at initial recognition, if any;
- b) a group of contracts that at initial recognition have no significant possibility of becoming onerous subsequently, if any; and
- c) a group of the remaining contracts in the portfolio, if any.

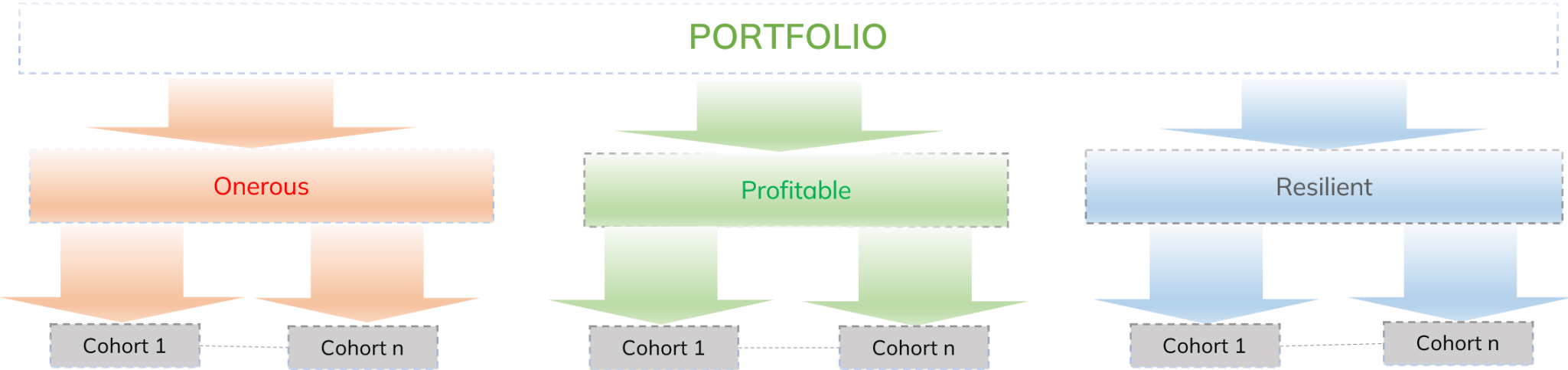
- 2.7. As stated in the excerpt above derived from the IFRS 17 guidance, SHA will subdivide its portfolio of insurance contracts as follows: -

| Profitability Group                       | SHA Interpretation                                                                              |
|-------------------------------------------|-------------------------------------------------------------------------------------------------|
| <b>Onerous Group</b>                      | Contracts where expected service costs exceed contributions (e.g., high-risk vulnerable groups) |
| <b>No Significant Risk of Onerousness</b> | Stable contributor groups with low adverse selection risk (e.g., formal sector)                 |
| <b>Other Contracts ('Resilient')</b>      | Other contributors with some uncertainty but positive expected margin                           |

### *c) Consideration of Contract Issuance Dates, i.e., breakdown of Cohorts*

- 2.8. Contracts within the same group will have the same underwriting year (not more than one year apart).
- 2.9. Due to the short-term nature of contracts issued by SHA, all products will be measured using the Premium Allocation Approach ("PAA").

Grouping Process





## 3. AGGREGATION OF INSURANCE CONTRACTS

### Initial Recognition

- 3.1. At the time of initial recognition, SHA will evaluate each contract to determine whether it is onerous, profitable, or “resilient”.
- 3.2. A contract is classified as onerous if the present value of expected costs exceeds the expected contributions, indicating a potential deficit.
- 3.3. A contract is considered profitable if expected contributions exceed the present value of expected costs, resulting in a surplus.
- 3.4. Contracts that do not fall into either of the above categories are considered resilient, meaning the expected contributions are in line with the expected costs, indicating a balanced and sustainable position over the coverage period.
- 3.5. However, given SHA’s public mandate, which enforces a uniform contribution structure, calculated as a percentage of income for the formal sector and a fixed amount for the informal sector, SHA cannot adjust contributions based on risk or expected utilisation.
- 3.6. As such, profitability groupings will reflect members’ relative earning capacity and healthcare usage patterns.
- 3.7. Members with higher contributions and lower expected utilisation will generally be classified as profitable, while those with lower contributions or higher expected utilisation, may be classified as onerous.

### Portfolios of Insurance Contracts

- 3.8. The classification of contracts into portfolios will consider contracts that are subject to similar risk and managed together within their respective funds.
- 3.9. Additionally, the classification will consider the nature and design of benefits and contribution mechanisms.
- 3.10. SHA will define portfolios based on its four underlying funds as follows:

| Portfolio                                       | Fund Basis                          | Characteristics                                                                                                                              |
|-------------------------------------------------|-------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Primary Healthcare Portfolio</b>             | Primary Healthcare Fund             | High-frequency, low-cost services; outpatient and preventive care; low-severity risk                                                         |
| <b>Social Health Insurance Portfolio</b>        | Social Health Insurance Fund        | Comprehensive inpatient and outpatient coverage; broader population risk; employer and voluntary contributions                               |
| <b>Emergency and Chronic Care Portfolio</b>     | Emergency and Chronic Illness Fund  | Low-frequency, high-severity risks; includes long-term conditions and emergencies; significant cost volatility                               |
| <b>Public Officers Medical Scheme Portfolio</b> | Public Officers Medical Scheme Fund | Tailored medical cover for public officers; structured employer-supported contributions; enhanced benefits and specialized healthcare access |

### Grouping within Portfolios

#### Onerous Contracts

- 3.11. A group of insurance contracts will be deemed onerous if the discounted present value of the expected costs surpasses the expected contributions. According to the IFRS 17 Standard paragraph 47;

An insurance contract is onerous at the date of initial recognition if the fulfilment cash flows allocated to the contract, any previously recognised acquisition cash flows and any cash flows arising from the contract at the date of initial recognition in total are a net outflow.

- 3.12. SHA shall group such contracts separately from those that are not onerous.
- 3.13. The assessment will consider expected healthcare services, contributions and subsidies received, and utilization patterns. As SHA does NOT incur acquisition costs, the fulfilment cash flows will exclude such amounts in this assessment.
- 3.14. The onerous group will constitute contracts where healthcare usage exceeds the contributions received.
- 3.15. SHA will identify onerous contracts by measuring a set of contracts rather than individual contracts.
- 3.16. SHA must record a loss in the profit and loss statement for the group of contracts that are onerous.

#### Profitable Contracts

- 3.17. These insurance contracts are anticipated to produce profitable results throughout the course of their duration. This will majorly include high contributing individuals with low utilisation.
- 3.18. These contracts may be divided further into groups that reflect different levels of profitability which will be guided by SHA's internal reporting system.

#### "Resilient" Contracts

- 3.19. Since not all contracts are considered onerous or profitable, the entity may aggregate contracts into a third group where the contracts are considered "resilient".
- 3.20. As per the Standard;

An entity is to assess a set of contracts to determine if the contracts are onerous and assess further to determine if the contracts have no significant possibility of becoming onerous.

- 3.21. Such contracts will then be grouped together if SHA has reasonable and supportable information to conclude that they have no significant possibility of becoming onerous.

#### Additional Considerations for Aggregation of Insurance Contracts

- 3.22. Each fund's characteristics will be examined further to determine whether contracts under that fund meet the definition of insurance contracts under IFRS 17.
- 3.23. As per paragraph B17 and B18 of the Standard;

A contract is an insurance contract only if it transfers significant insurance risk.

Insurance risk is significant if, and only if, an insured event could cause the issuer to pay additional amounts that are significant in any single scenario, excluding scenarios that have no commercial substance (i.e. no discernible effect on the economics of the transaction). If an insured event could mean significant additional amounts would be payable in any scenario that has commercial substance, the condition in the previous sentence can be met even if the insured event is extremely unlikely, or even if the expected (i.e. probability-weighted) present value of the contingent cash flows is a small proportion of the expected present value of the remaining cash flows from the insurance contract.

- 3.24. Contracts within each portfolio will only be included if they meet the above criteria.
- 3.25. Contracts that fall outside the scope of IFRS 17 shall be accounted for in accordance with the guidance provided for under the "Scope" accounting policy paper.

## 4. FURTHER DISAGGREGATION

- 4.1. As per IFRS 17, an entity is allowed to perform further disaggregation of insurance contracts. The additional disaggregation may entail segmenting the contracts into more precise groups based on specific criteria.
- 4.2. This additional disaggregation may be used to analyse and oversee contracts on a more granular level thus ensuring accurate financial reporting that reflects the specific risks tied to each subset of contracts.
- 4.3. If there are contracts within a portfolio that fall into different groups solely because legal or regulatory constraints prevent the entity from setting different prices or benefit levels for policyholders with different characteristics, the entity may include those contracts in the same group.

### Criteria for further disaggregation

- 4.4. The entity may perform further disaggregation of the groups within portfolios as follows:
  - Separate non-onerous groups at initial recognition based on internal reporting that distinguishes:
    - Different profitability levels, or
    - Different risks of becoming onerous after initial recognition.
  - Create multiple groups for contracts that are onerous at initial recognition if internal reporting provides detailed information on the extent of their onerousness.
- 4.5. SHA will however, **NOT** perform a further disaggregation on its insurance contracts.

## 5. MEASUREMENT MODELS

- 5.1. Under IFRS 17, three measurement models are prescribed for insurance contracts:
- a) **General Measurement Model (GMM)**, the default approach, measures the CSM by estimating future cash flows, adjusting for risk and time value of money.
  - b) **The Variable Fee Approach (VFA)** is a variation of GMM, used for direct participating contracts where the insurer's return is a share in the fair value of underlying items.
  - c) **The Premium Allocation Approach (PAA)** is a simplified model permitted for short-duration contracts, where measurement resembles the unearned premium reserve method under previous standards.
- 1.2 For SHA, VFA shall not apply because the insurance contracts under consideration do not involve direct participation features.
- 1.3 Due to the short-term nature of the contracts issued by SHA, aggregation of contracts will be done using the Premium Allocation Approach (PAA).
- 5.2. Under the PAA, the initial measurement only reflects the contributions received since SHA does not incur any acquisition costs. Subsequent measurement adjusts for earned contributions and any changes in estimates of future cash flows.

### Premium Allocation Approach (PAA)

- 5.3. Paragraph 18 of IFRS 17 states;

For contracts issued to which an entity applies the premium allocation approach, the entity shall assume no contracts in the portfolio are onerous at initial recognition, unless facts and circumstances indicate otherwise. An entity shall assess whether contracts that are not onerous at initial recognition have no significant possibility of becoming onerous subsequently by assessing the likelihood of changes in applicable facts and circumstances.

- 5.4. The entity will apply PAA for **ALL** its insurance contracts.
- 5.5. For such contracts measured under PAA, the insurer will assume that no contracts in the portfolio are onerous at initial recognition unless facts and circumstances indicate otherwise, in this case where the healthcare usage exceeds the contributions received.
- 5.6. For contracts found to be onerous based on the above-mentioned facts and circumstances, a loss component will be established as part of the liability for remaining coverage (LRC) depicting the losses recognized.
- 5.7. The loss component amounts will be presented in the profit or loss as reversals of losses on the onerous groups and shall not be included in determination of the insurer's insurance revenue. These reversals should be made systematically and proportionately based on the coverage units provided during the reporting period, reflecting the service delivered.
- 5.8. For the contracts that are not onerous at initial recognition, the insurer shall assess whether the contracts have any significant possibility of becoming onerous subsequently by assessing the likelihood of changes in applicable facts and circumstances.

## 6. RELEVANT IFRS 17 TEXT

Refer to the IFRS 17 Standard available online at the address below:

<https://www.ifrs.org/content/dam/ifrs/publications/pdf-standards/english/2022/issued/part-a/ifrs-17-insurance-contracts.pdf?bypass=on>

The policy has been adopted and signed by the Board of Management of the Social Health Authority on this \_\_\_\_\_ day of \_\_\_\_\_ 2026.

Chief Executive Officer:

\_\_\_\_\_

Signature:

\_\_\_\_\_

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## Document Version

May 2026

Version: 2.0 – Level of Aggregation

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